QLABS DIAGNOSTICS

Patient ID	: 10125/5867	Sample ID	: 2505437
Name	**: MRS FATIMA**	Registration Date	: 09/05/2025 01:51 PM
Age/Gender	: 69 Yrs/Female	Report Date	: 10/05/2025 09:34 AM
Ref. Dr.	: Dr. MD MUBASHEER ALI	10125/5867	
Center	: TRUE CARE CLINIC		

HAEMATOLOGY REPORT

Test Description	Result	Unit	Biological Reference Range
COMPLETE BLOOD PICTURE			
Hemoglobin	12.2	gm/dL	11.0 - 15.0
RBC Count	4.5	mil/cu.mm	3.8 - 5.5
Hematocrit HCT	37.6	%	36 - 48
Platelet Count	3.64	lac/cmm	1.5 - 4.5
Red Cell Indices			
Mean Corp Volume MCV	83.6	fL	83 - 101
Mean Corp Hb MCH	27.1	pg	27 - 32
Mean Corp Hb Conc MCHC	32.4	gm/dL	31.5 - 34.5
RDW-CV	**14.3**	%	11.6 - 14.0
Total Count and DIFFERENTIAL LEUCOCYTE COUNT			
Total WBC Count	**13400**	cell/cu.mm	4000 - 10000
Neutrophils	69	%	40 - 75
Lymphocytes	25	%	20 - 40
Eosinophils	02	%	01 - 06
Monocytes	04	%	02 - 10
Basophils	00	%	00 - 02
Microscopic Blood Picture			
RBC Morphology	Normocytic Normochromic		
WBC Morphology	**Leukocytosis**		
Platelet on smear	Adequate		

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ESR - ERYTHROCYTE SEDIMENTATION RATE	**76**	mm/hr	0 - 22

Method : Automated Westergren

INTERPRETATION :

1. It indicates presence and intensity of an inflammatory process, never diagnostic of a specific disease. Changes are more significant than a single abnormal test.
2. It is a prognostic test and used to monitor the course or response to treatment of diseases like tuberculosis, bacterial endocarditis, acute rheumatic fever, rheumatoid arthritis, SLE, Hodgkins disease, temporal arteritis, polymyalgia rheumatica.
3. It is also increased in pregnancy, multiple myeloma, menstruation, and hypothyroidism.

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BIOCHEMISTRY REPORT

Test Description	Result	Unit	Biological Reference Range
KIDNEY FUNCTION TEST(KFT)			
Serum Creatinine *Method: Modified Jaffe with no deproteinzation*	1.0	mg/dL	0.60 - 1.40
Urea *Method: Urease UV GLDH*	18	mg/dl	10.0 - 45.0
Blood Urea Nitrogen-BUN *Method: Calculated*	7.2	mg/dl	7 - 20
Uric Acid *Method: Uricase Peroxidase*	4.7	mg/dl	2.5 - 6.8
Sodium *Method: ISE*	142	mEq/L	135 - 146
Potassium *Method: ISE*	4.0	mEq/L	3.5 - 5.5
Chloride	99	mEq/L	96.0 - 109

Interpretation :

Kidney function tests help to screen the individual for renal disease and to determine the extent or progression of renal disease. These tests also aid in determining drug dosage for drugs excreted through the kidneys. The clinical syndrome resulting from decreased renal function and azotemia is called uremia Renal azotemia: glomerular nephritis and chronic pyelonephritis. Prerenal azotemia: severe dehydration, hemorrhagic shock, and excessive protein intake. Post renal azotemia: urethral stones or tumors and prostatic obstructions Measurement of urea in dialysis fluids is widely used in assessing the adequacy of renal replacement therapy.

In these prerenal situations, the plasma creatinine concentration may be normal.In obstructive post renal conditions, both plasma creatinine and urea concentrations will be increased, although there is often a greater increase in plasma urea than creatinine because of the increased back diffusion.These considerations give rise to the principal clinical utility of plasma urea, which lies in its measurement in conjunction with that of plasma creatinine and subsequent calculation of the urea nitrogen/creatinine ratio. This ratio has been used as a crude discriminator between prerenal and postrenal azotemia. Significantly lower ratios usually denote (1) acute tubular necrosis, (2) low protein intake, (3) starvation, or (4) severe liver disease (decreased urea synthesis). So even though blood urea is not an excellent marker of renal dysfunction as it rises quite late in the dysfunction and its rise is also not exclusive to kidney dysfunction, but for practical purposes serum urea level is still one of the most ordered test and forms an important part of the kidney function test.

Concentrations in excess of 6.0 mg/dL at 32 weeks gestation have been noted to be associated with a high perinatal mortality rate.

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LIVER FUNCTION TEST (LFT)

TOTAL BILIRUBIN	**1.26**	mg/dl	0.1 -1.2
Method: Serum, Jendrassik Grof			
DIRECT BILIRUBIN	**0.39**	mg/dL	0.0 -0.3
Method: Serum, Diazotization			
INDIRECT BILIRUBIN	0.87	mg/dl	0.0 -1.0
Method: Serum, Calculated			
SGPT (ALT)	14.28	U/L	0 - 49
Method: Serum, UV with P5P, IFCC 37 degree			
SGOT (AST)	21.07	U/L	0 - 46
Method: Serum, UV with P5P, IFCC 37 degree			
ALKALINE PHOSPHATASE	124.15	U/L	42 - 125
Method: DGKC			
TOTAL PROTEIN	6.88	g/dl	6.0 - 8.3
Method: Serum, Biuret, reagent blank end point			
SERUM ALBUMIN	4.05	g/dl	3.4 - 5.4
Method: Serum, Bromocresol green			
SERUM GLOBULIN	2.83	g/dl	1.8 - 3.6
Method: Serum, Calculated			
A/G RATIO	1.43		1.2 - 2.2
Method: Serum, Calculated			

NOTE :

In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or Non alcoholic fatty liver disease, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

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SEROLOGY & IMMUNOLOGY REPORT

Test Description	Result	Unit	Biological Reference Range
CRP (C-REACTIVE PROTEIN)	**50.8**	mg/L	0 - 6

Method: Immunoturbidometry

Test Usage:

Determine the presence of C-reactive protein. Nonspecific, but most sensitive indicator of inflammation and/or necrosis. A baseline presurgical CRP may be useful in determining postoperative infection.

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IMMUNOASSAY REPORT

Test Description	Result	Unit	Biological Reference Range
CARCINOEMBRYONIC ANTIGEN	2.60	ng/ml	0 - 5.2

Method: CLIA

Method : ECLIA

Interpretation :

1. CEA (Carcinoembryonic Antigen), is an oncofetal glycoprotein and is expressed in normal mucosal cells and over expressed in adenocarcinoma, especially colorectal cancer.

2. CEA is used as a marker for monitoring colorectal and gastrointestinal carcinoma and is raised in carcinoma of lung, breast,liver,pancreas, prostate ,stomach and ovary .

3. Benign conditions which can elevate CEA include smoking, hepatic diseases, infections, inflammatory bowel disease, trauma, collagen vascular disease, renal disorders, pancreatitis, cirrhosis of the liver and peptic ulcer, hypothyroidism, chemotheraphy and radiation. Although values are usually less than 10 ng/mL.

4. CEA is not an effective screening test for hidden (occult cancer since early tumors do not cause significant blood elevations .

5. A single test result is difficult to evaluate, but a number of tests, done weeks apart, shows trends in disease progression or regression.

CA-125-Serum	5.48	U/ml	0 - 35

Method : ECLIA

Clinical significance

1) CA 125 is a glycoprotein normally expressed in coelomic epithelium, which lines body cavities and envelopes the ovaries.

2. CA 125 levels are elevated in about 85 percent of women with ovarian cancer (especially serous epithelial tumours), but in only 50 percent of those with stage I disease.

3. Multiple benign disorders like Menstruation, pregnancy, fibroids, ovarian cysts, pelvic inflammation, cirrhosis, ascites, pleural and pericardial effusions, endometriosis also are associated with CA 125 elevations.

4. Levels above which benign diseases are considered unlikely are 200U/ml in premenopausal & 35 u/ml for postmenopausal women

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AFP (Alpha Fetoprotein)-Serum	2.88	ng/ml	Pregnancy(median) 14W:<27.9 15W:<30.9 16W:<36.1 17W:<40.4 18W:<48.3 19W:<54.8

Method : ECLIA

COMMENT:

Alpha-fetoprotein (AFP) is synthesized primarily in liver and yolk sac of the fetus. AFP is elevated in the serum of more than 70% of patients with hepatocellular carcinoma and 70% of patients with non-seminomatous testicular carcinoma. It is also elevated during pregnancy and also in association with gastro-intestinal cancers with or without liver metastasis. Postoperative AFP values which fail to return to normal strongly suggest the presence of residual tumor.

The use of AFP as a tumor marker in pregnant females is not recommended.

Values obtained from different assay methods cannot be used interchangeably. AFP levels, regardless of value, should not be interpreted as absolute evidence of the presence or absence of disease.

PREGNANCY:

Maternal AFP values are used to screen for neural tube defects (NTDs). The values depend on gestational age and are expressed as MOM (multiples of median) in order to facilitate comparison of AFP test results across gestational weeks and between laboratories.

Increase of greater than 2.5 MOM can occur in open NTDs such as spina bifida and anencephaly. AFP may also be increased in omphalocele, cystic hygroma and congenital nephrosis.

Less than 0.5 MOM is seen in 5% of unaffected pregnancies and in about 20% of Down's syndrome pregnancies. Decreased levels of maternal AFP are also present in Trisomy 18, long standing fetal death, maternal diabetes, and obesity, overestimation of gestational age, hydatidiform mole and choriocarcinoma.

Gestation age in weeks	Normal values (ng/ml)	Median ng/dl
14 weeks	6.0 - 52.8	27.9
15 weeks	8.4 - 61.2	30.9
16 weeks	8.4 - 72.0	36.1
17 weeks	12.0 - 84.0	40.4
18 weeks	14.4 - 94.8	48.3
19 weeks	16.8 - 108.0	54.8
20 week	16.8 - 129.0	60.9
21 weeks	19.2 - 163.2	70.10
22 weeks	24.0 - 182.4	80.64

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CA-19.9-Serum	12.50	U/ml	0 - 35

Method : ECLIA

Clinical Significance

CA 19-9 - A large percentage of patients with gastrointestinal tumors (such as pancreatic, liver, gastric, colorectal tumors) and some other malignancies have been shown to have elevated serum CA 19-9 levels. Serum CA 19-9 levels may be useful for monitoring disease activity or predicting relapse following treatment. CA 19-9 should not be used as a screening test.

**** End of the report. ****
Please correlate clinically.

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